

ASADIN Injection 1.0mg/mL (Arsenic Trioxide)

{DESCRIPTION}

ASADIN is formulated as a clear solution of Arsenic Trioxide. Each mL contains 1.0mg of Arsenic Trioxide.

{MECHANISM OF ACTION}

The mechanism of action of Arsenic Trioxide is not completely understood. In vitro studies, Arsenic Trioxide causes morphological changes and DNA fragmentation characteristic of apoptosis in NB4 human promyelocytic leukemia cells. Arsenic Trioxide also causes damage or degradation of the fusion protein PML- RAR α .

{PHARMACOKINETICS}

The pharmacokinetics of Arsenic Trioxide have not been characterized.

Metabolism

The metabolism of Arsenic Trioxide involves reduction of pentavalent arsenic to trivalent arsenic by arsenate reductase and methylation of trivalent arsenic to monomethylarsonic acid and monomethylarsonic to dimethylarsinic acid by methyltransferases. The main site of methylation reactions appears to be the liver. Arsenic is stored mainly in liver, kidney, heart, lung, hair and nails.

Excretion

Disposition of arsenic following intravenous administration has not been studied. Trivalent arsenic is mostly methylated in humans and excreted in urine.

Special Populations

The effects of renal or hepatic impairment or gender, age and race on the pharmacokinetics of Arsenic Trioxide have not been studied.

Drug Interactions

No formal assessments of pharmacokinetic drug-drug interactions between Arsenic Trioxide and other drugs have been conducted. The methyltransferases responsible for metabolizing Arsenic Trioxide are not members of the cytochrome P450 family of isoenzymes.

Clinical Studies

In an open-label, single-arm, non-comparative study, Arsenic Trioxide has been investigated in 40 relapsed or refractory acute promyelocytic leukemia (APL) patients, previously treated with an anthracycline and a retinoid regimen. Patients received Arsenic Trioxide 0.15mg/kg/day intravenously over 1 to 2 hours until the bone marrow was cleared of leukemic cells or up to a maximum of 60 days. The complete response (CR) (absence of visible leukemic cells in bone marrow and peripheral recovery of platelets and white blood cells with a confirmatory bone marrow 30 days later) rate in this population of previously treated patients was 28 of 40 (70%). Among the 22 patients who had relapsed less than 1 year after treatment with all-trans retinoic acid (ATRA), there were 18 complete responders (82%). Of the 18 patients receiving Arsenic Trioxide a 1 year from ATRA treatment, there were 10 complete responders (55%). The median time to bone marrow remission was 44 days and to onset of CR was 53 days. 3 of 5 children, 5 years or older, achieved CR. No children less than 5 years old were included in this study.

3 to 6 weeks following bone marrow remission, 31 patients received consolidation therapy with Arsenic Trioxide, at the same dose, for 25 additional days over a period up to 5 weeks. In follow-up treatment, 18 patients received further Arsenic Trioxide as a maintenance course. 15 patients had bone marrow transplants. At last follow-up, 27 of 40 patients were alive with a median follow-up time of 484 days (range 280 to 755 days), and 23 of 40 patients remained in complete response with a median survival time of 483 days (range 280 to 755 days). Cytogenetic conversion to no detection of the APL chromosome rearrangement was observed in 24 of 28 (86%) patients who met the response criteria defined above, in 5 of 5 (100%) patients who met some but not all of the response criteria, and in 3 of 7 (43%) refractory patients. Reverse Transcriptase-Polymerase Chain Reaction conversions to no detection of the APL gene rearrangement were demonstrated in 22 of 28 (79%) patients who met the

response criteria, in 3 of 5 (60%) patients who met some but not all of the response criteria, and in 2 of 7 (29%) of refractory patients who did not respond.

Responses were seen across all age groups tested, ranging from 6 to 72 years. The ability to achieve a CR was similar for both genders. There were insufficient patients of Africans, Hispanic or Asian derivations to estimate relative response rates in these groups, but responses were seen in members of each group.

Another study in 12 patients with relapsed or refractory APL, where patients received Arsenic Trioxide doses generally similar to the recommended dose, had results with 9 (75%) patients attaining a CR.

[INDICATIONS]

1) Newly Diagnosed Low to Intermediate Risk Acute promyelocytic leukaemia

Arsenic Trioxide is indicated in combination with tretinoin for treatment of adults with newly diagnosed low to intermediate risk acute promyelocytic leukemia (APL) whose APL is characterized by the presence of the t(15;17) translocation or PML/RAR alpha gene expression.

2) Relapsed or Refractory Acute promyelocytic leukaemia

For the induction of remission and consolidation in patients with acute promyelocytic leukaemia (APL) who are refractory to, or have relapsed from, retinoid and anthracycline chemotherapy, and whose APL is characterised by the presence of the t(15;17) translocation or PML/RAR alpha gene expression.

[DOSAGE AND ADMINISTRATION]

Use under physician's prescription only.

For intravenous infusion, Arsenic Trioxide should be diluted with 100 to 500 ml 5% Dextrose or 0.9% Sodium Chloride injection, and administered intravenously over 2 - 4 hours. The infusion duration may be extended up to 4 hours if acute vasomotor reactions are observed.

1) Recommended Dosage for Newly Diagnosed Low to Intermediate Risk Acute Promyelocytic Leukemia (APL)

A treatment course for patients with newly diagnosed low to intermediate risk APL consists of 1 induction cycle and 4 consolidation cycles.

- For the induction cycle, the recommended dosage of Arsenic Trioxide is 0.15 mg/kg intravenously daily in combination with tretinoin until bone marrow remission but not to exceed 60 days (see Table 1).
- For the consolidation cycles, the recommended dosage of Arsenic Trioxide is 0.15 mg/kg intravenously daily 5 days per week during weeks 1- 4 of each 8 week cycle for a total of 4 cycles in combination with tretinoin (see Table 1). Omit tretinoin during weeks 5 -6 of the fourth cycle of consolidation.

Table 1 Recommended Dose of ASADIN in Low to Intermediate Risk Patients

Induction (1 cycle)

Arsenic Trioxide 0.15 mg/kg once daily intravenously	Until marrow remission but not to exceed 60 days
Tretinoin ^a 22.5 mg/m ² twice daily orally	Until marrow remission but not to exceed 60 days

Consolidation (4 cycles)

	Week							
	1	2	3	4	5	6	7	8
Arsenic Trioxide 0.15 mg/kg QD , iv	D ^c 1- 5	D 1-5	D 1-5	D 1-5	--	--	--	--
Tretinoin ^a	D	D	--	--	D ^b	D ^b	--	--

22.5 mg/m ² BID, po	1-7	1-7			1-7	1-7		
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^aRounded to the nearest 10 mg increment

^bOmitted during the 4th cycle of consolidation

^cD: Days

Differentiation syndrome prophylaxis consisting of prednisone 0.5 mg/kg daily from day 1 until the end of induction therapy with Arsenic Trioxide and tretinoin is recommended.

2) Recommended Dosage for Relapsed or Refractory APL

Induction Treatment Schedule

Arsenic Trioxide should be administered intravenously at a dose of 0.15mg/kg/day until bone marrow remission. Total induction dose should not exceed 60 doses.

Consolidation Treatment Schedule

Consolidation treatment should begin 3 to 6 weeks after completion of induction therapy. Arsenic Trioxide should be administered intravenously at a dose of 0.15mg/kg/day.

3) Monitoring and Dosage Modifications for Adverse Reactions

During induction monitor coagulation studies, blood counts, and chemistries at least 2-3 times per week through recovery. During consolidation, monitor coagulation studies, blood counts, and chemistries at least weekly.

Table 2 shows the dose modifications for adverse reactions due to Arsenic Trioxide when used alone or in combination with tretinoin.

Table 2 Dose Adjustments for Adverse Reactions of Arsenic Trioxide

Adverse Reaction	Dose Modification
Differentiation syndrome, defined by the presence of 2 or more of the following: - Unexplained fever - Dyspnea - Pleural and/or pericardial effusion - Pulmonary infiltrates - Renal failure - Hypotension - Weight gain greater than 5 kg [see Warnings and Precautions]	• Temporarily withhold ASADIN. Consider holding tretinoin if symptoms are severe. • Administer dexamethasone 10 mg intravenously every 12 hours until the resolution of signs and symptoms for a minimum of 3 days. • Resume treatment when the clinical condition improves and reduce the dose of the withheld drug(s) by 50%. • Increase the dose of the withheld drug(s) to the recommended dosage after one week in the absence of recurrence of symptoms of differentiation syndrome. • If symptoms re-appear, decrease ASADIN and/or tretinoin to the previous dose.
QTc (Framingham formula) Prolongation greater than 450 msec for men or greater than 460 msec for women [see Warnings and Precautions]	• Withhold Arsenic Trioxide and any medication known to prolong the QTc interval. • Correct electrolyte abnormalities. • After the QTc normalizes, and electrolyte abnormalities are corrected, resume treatment with ASADIN at a 50% reduced dose (0.075 mg/kg daily) for one week after resolution. • If the 50% reduced dose is tolerated for

	one week (in the absence of QTc prolongation), increase the dose of ASADIN to 0.11 mg/kg /day for the next week [see Dosage and Administration] • The dose of ASADIN can be increased to 0.15 mg/kg /day in the absence of QTc prolongation during that 14-day dose-escalation period.						
Hepatotoxicity, defined by 1 or more of the following: - Total bilirubin (TB) greater than 3 times the upper limit of normal (ULN) - Aspartate aminotransferase (AST) greater than 5 times the ULN - Alkaline phosphatase (AP) greater than 5 times the ULN [see Warnings and Precautions]]	• Withhold ASADIN and/or tretinoin. • Resume treatment at a 50% reduced dose of the withheld drug(s) when TB is less than 1.5 times the ULN and AP/AST are less than 3 times the ULN • Increase the dose of the withheld drug(s) back to the recommended dosage after one week on the reduced dose in the absence of worsening of hepatotoxicity • Discontinue the withheld drug(s) permanently if hepatotoxicity recurs.						
Other severe or life-threatening (grade 3-4) nonhematologic reactions [see Adverse Reactions]	• Temporarily withhold ASADIN and tretinoin. • When the adverse reaction resolves to no more than mild (grade 1), resume ASADIN and tretinoin reduced by 2 dose levels (see Table 3 below).						
Moderate (grade 2) nonhematologic reactions[see Adverse Reactions (6)]	• Reduce the dose of ASADIN and/or tretinoin by 1 dose level (see Table 3 below).						
Leukocytosis (WBC count greater than 10Gi/L) [see Adverse Reactions]	• Administer hydroxyurea <table border="1"> <tr> <th>WBC</th><th>Hydroxyurea</th></tr> <tr> <td>10-50Gi/L</td><td>500 mg QID</td></tr> <tr> <td>>50Gi/L</td><td>1000mg QID</td></tr> </table> • Hydroxyurea may be discontinued when the WBC declines below 10 Gi/L.	WBC	Hydroxyurea	10-50Gi/L	500 mg QID	>50Gi/L	1000mg QID
WBC	Hydroxyurea						
10-50Gi/L	500 mg QID						
>50Gi/L	1000mg QID						
Myelosuppression, defined by 1 or more of the following: - absolute neutrophil count less than 1 Gi/L - platelets less than 50 Gi/L lasting more than 5 weeks [see Adverse Reactions]	• Consider reducing the dose of ASADIN and tretinoin by1 dose level (see Table 3 below). • If myelosuppression lasts ≥ 50 days or occurs on 2 consecutive cycles, assess a marrow aspirate for remission status. In the case of molecular remission, resume ASADIN and tretinoin at 1 dose level lower (see Table 3 below).						

Table 3 Dose Reduction Levels for Hematologic and Nonhematologic Toxicities

Dose Level	Arsenic Trioxide mg/kg intravenously once daily	Tretinoin* mg/m² orally twice daily
Starting level	0.15	22.5
-1	0.11	18.75
-2	0.10	12.5
-3	0.075	10

*Rounded to the nearest 10 mg increment

[CONTRAINDICATIONS]

ASADIN is contraindicated in patients who are hypersensitive to arsenic and any of the excipients.

[WARNINGS]

1. ASADIN should be administered under the supervision of a physician who is experienced in the management of patients with acute leukemia.

2. APL Differentiation Syndrome:

Based on the report, 9 of 40 patients with APL treated with Arsenic Trioxide, at a dose of 0.15mg/kg, experienced the APL differentiation syndrome.

Some patients with APL treated with Arsenic Trioxide have experienced symptoms similar to a syndrome called the retinoic- acid-Acute Promyelocytic Leukemia (RA-APL) or APL differentiation syndrome, characterized by fever, dyspnea, weight gain, pulmonary infiltrates and pleural or pericardial effusions, with or without leukocytosis. This syndrome can be fatal. The management of the syndrome has not been fully studied, but high- dose steroids have been used at the first suspicion of the APL differentiation syndrome and appear to mitigate signs and symptoms. At the first signs that could suggest the syndrome (unexplained fever, dyspnea and/or weight gain, abnormal chest auscultatory findings or radiographic abnormalities), high-dose steroids (dexamethasone 10mg intravenously BID) should be immediately initiated, and continued for at least 3 days or longer until symptoms have abated. The majority of patients do not require termination of Arsenic Trioxide therapy during treatment of the APL differentiation syndrome.

3.ECG Abnormalities

Arsenic Trioxide can cause QT interval prolongation and complete atrioventricular block. QT prolongation can lead to a torsade de pointes-type ventricular arrhythmia, which can be fatal. The risk of torsade de pointes is related to the extent of QT prolongation, concomitant administration of QT prolonging drugs, a history of torsade de pointes, preexisting QT interval prolongation, congestive heart failure, administration of potassium-wasting diuretics, or other conditions that result in hypokalemia or hypomagnesemia. Base on the overseas report, one patient (also receiving amphotericin B) had torsade de pointes during the therapy for relapsed APL with Arsenic Trioxide.

4. ECG and Electrolyte Monitoring Recommendations

Base on the overseas report, prior to initiating therapy with Arsenic Trioxide, a 12-lead ECG should be performed and serum electrolytes (potassium, calcium, and magnesium) and creatinine should be assessed; preexisting electrolyte abnormalities should be corrected and, if possible, drugs that are known to prolong the QT interval should be discontinued. For QTc > 500 msec, corrective measures should be completed and the QTc reassessed with serial ECGs prior to considering using Arsenic Trioxide. During therapy with Arsenic Trioxide, potassium concentrations should be kept above 4 mEq/dL and magnesium concentrations should be kept above 1.8 mg/dL. Patients who reach an absolute QT interval value > 500 msec should be reassessed and immediate action should be taken to correct concomitant risk factors, if any, while the risk/benefit of continuing versus suspending Arsenic Trioxide therapy should be considered. If syncope, rapid or irregular heartbeat develops, the patient should be hospitalized for monitoring, serum electrolytes should be assessed, Arsenic Trioxide therapy should be temporarily discontinued until the QTc interval regresses to below 460 msec, electrolyte abnormalities are corrected, and the syncope and irregular heartbeat

cease. There are no data on the effect of Arsenic Trioxide on the QTc interval during the infusion.

5. Hyperleukocytosis

Based on the report, treatment with Arsenic Trioxide has been associated with the development of hyperleukocytosis ($\geq 10 \times 10^3/\mu\text{L}$) in 20 of 40 patients. A relationship did not exist between baseline WBC counts and development of hyperleukocytosis nor baseline WBC counts and peak WBC counts. Hyperleukocytosis was not treated with additional chemotherapy. WBC counts during induction treatment were higher than during consolidation treatment.

6. QT Prolongation

Based on the report, QT/QTc prolongation should be expected during treatment with Arsenic Trioxide and torsade de pointes as well as complete heart block has been reported. Over 460 ECG tracings from 40 patients with refractory or relapsed APL treated with Arsenic Trioxide were evaluated for QTc prolongation. 16 of 40 patients (40%) had at least one QTc interval greater than 500 msec between 1 to 5 weeks after Arsenic Trioxide infusion, and then returned toward baseline by the end of 8 weeks after treatment. In these ECG evaluations, there were no gender differences, and there was no correlation with age.

7. Complete AV Block

Complete AV block has been reported with Arsenic Trioxide in one case of a patient with APL.

[PRECAUTIONS]

1. Treatments with Arsenic Trioxide may develop hyperleukocytosis, monitoring should be performed.

2. Administration of Arsenic Trioxide should be discontinued if thoracic-abdominal effusion appears during therapy.

3. Laboratory Tests

The patient's electrolyte and hematologic profiles should be monitored at least twice weekly. Clinically unstable patients should be monitored more frequently during the induction treatment and at least weekly during the consolidation treatment. ECGs should be monitored weekly, and more frequently for clinically unstable patients, during induction treatment and consolidation treatment.

4. Drug Interactions

Caution is advised when Arsenic Trioxide is coadministered with other medications that can prolong the QT interval (e.g. antiarrhythmics or thioridazine) or lead to electrolyte abnormalities (e.g. diuretics or amphotericin B).

5. Carcinogenesis, Mutagenesis, Impairment Of Fertility

Based on the overseas studies, Arsenic Trioxide is a human carcinogen. Arsenic Trioxide and trivalent arsenite salts have not been demonstrated to be mutagenic to bacteria, yeast or mammalian cells. In vitro study, that arsenite salts are clastogenic (human fibroblasts, human lymphocytes, Chinese hamster ovary cells, Chinese hamster V79 lung cells). Trivalent arsenic produced an increase in the incidence of chromosome aberrations and micronuclei in bone marrow cells of mice. The effect of arsenic on fertility has not been adequately studied.

6. Pregnancy

Pregnancy Category D

Arsenic Trioxide is contraindicated in pregnant women. Women of childbearing potential should be advised to avoid becoming pregnant.

Based on the study reports, Arsenic Trioxide may cause fetal harm when administered to a pregnant woman. Studies in pregnant mice, rats, hamsters, and primates have shown that Arsenic Trioxide cross the placenta when given orally or by injection. The reproductive toxicity of Arsenic Trioxide has been studied in a limited manner. An increase in resorptions, neural-tube defects, anophthalmia and microphthalmia were observed in rats administered 10mg/kg of Arsenic Trioxide on gestation day 9 (approximately 10 times the recommended human

dose on a mg/m² basis). Similar findings occurred in mice administered a 10mg/kg dose of related trivalent arsenic, sodium arsenite (approximately 5 times the projected human dose on a mg/m² basis) on gestation days 6, 7, 8 or 9. Intravenous injection of 2mg/kg sodium arsenite (approximately equivalent to the projected human dose on a mg/m² basis) on gestation day 7 (the lowest dose tested) resulted in neural-tube defects in hamsters.

There are no reports in pregnant women using Arsenic Trioxide. If this drug is used during pregnancy or if the patient becomes pregnant while taking this drug, the patient should be apprised of the potential harm to the fetus. There is a report of one patient who became pregnant during therapy had a miscarriage.

7. Lactation

Arsenic Trioxide is excreted in human milk. Because of the potential for serious adverse reactions in nursing infants, an important decision should be made, whether to discontinue breastfeeding or to discontinue the drug.

8. Pediatric Use

There are limited clinical data on the pediatric use of Arsenic Trioxide. Based on the report, of 5 patients below the age of 18 years (5-16 years old) treated with Arsenic Trioxide, at the recommended dose of 0.15mg/kg/day, 3 achieved a complete response.

Safety and effectiveness in pediatric patients below the age of 5 years have not been studied.

9. Patients With Renal or Hepatic Impairment

Safety and effectiveness of Arsenic Trioxide in patients with renal and hepatic impairment have not been studied. Particular caution is needed in patients with renal failure receiving Arsenic Trioxide, because renal excretion is the main route of elimination of arsenic.

[ADVERSE REACTIONS]

Based on the studies, safety information was available for 52 patients with refractory or relapsed APL who participated in clinical trials of Arsenic Trioxide. Most patients experienced some drug-related toxicity, most commonly leukocytosis, gastrointestinal symptoms (nausea, vomiting, diarrhea, and abdominal pain), fatigue, edema, hyperglycemia, dyspnea, cough, rash or itching, headaches, and dizziness. These adverse effects have not been observed to be permanent or irreversible nor do they usually require interruption of therapy.

In the Phase 2 study of Arsenic Trioxide, with 40 patients with refractory or relapsed APL, the serious adverse events (grade 3 or 4 according to version 2 of the NCI Common Toxicity Criteria) included APL differentiation syndrome (n = 3), hyperleukocytosis (n = 3), QTc interval $\geq$ 500 msec (n = 16, 1 with torsade de pointes), atrial dysrhythmias (n = 2), and hyperglycemia (n = 2).

The following table describes the adverse events that were observed in these 40 patients treated for APL with Arsenic Trioxide at the recommended dose of 0.15mg/kg/day at a rate of 5% or more. Similar adverse event profiles were seen in the other patient populations who received Arsenic Trioxide.

Adverse Events (any grade) occurring in $\geq 5\%$ of 40 APL Patients with APL who received Arsenic Trioxide at a dose of 0.15mg/kg/day				
System organ class / Adverse Event	All Adverse Events, Any Grade		Grade 3 & 4 Events	
	n	%	n	%
General disorders and administration site conditions				
Fatigue	25	63	2	5
Pyrexia	25	63	2	5
Edema- non- specific	16	40		
Rigors	15	38		
Chest pain	10	25	2	5
Injection site pain	8	20		
Pain· non· specific	6	15	1	3
Injection site erythema	5	13		
Injection site edema	4	10		
Weakness	4	10	2	5
Hemorrhage	3	8		
Weight gain	5	13		
Weight loss	3	8		
Drug hypersensitivity	2	5	1	3
Gastrointestinal disorders				
Nausea	30	75		
Anorexia	9	23		
Appetite decreased	6	15		
Diarrhea	21	53		
Vomiting	23	58		
Lower & upper of abdominal pain	23	58	4	10
Sore throat	14	35		
Constipation	11	28	1	3
Loose stools	4	10		
Dyspepsia	4	10		
Oral blistering	3	8		
Fecal incontinence	3	8		
Gastrointestinal hemorrhage	3	8		
Dry mouth	3	8		
Abdominal tenderness	3	8		
Diarrhea hemorrhagic	3	8		
Abdominal distension	3	8		
Metabolism and nutrition disorders				
Hypokalemia	20	50	5	13
Hypomagnesemia	18	45	5	13
Hyperglycemia	18	45	5	13
ALT increased	8	20	2	5
Hyperkalemia	7	18	2	5
AST increased	5	13	1	3
Hypocalcemia	4	10		
Hypoglycemia	3	8		
Acidosis	2	5		
Nervous system disorders				
Headache	24	60	1	3
Insomnia	17	43	1	3
Paresthesia	13	33	2	5
Dizziness	9	23		
Tremor	5	13		

Eyelid edema	2	5		
Cardiac disorders				
Tachycardia	22	55		
ECG QT interval prolonged >500msec	16	40		
Palpitations	4	10		
ECG abnormal other than QT interval prolongation	3	8		
Infections				
Sinusitis	8	20		
Herpes simplex	5	13		
Upper respiratory tract infection	5	13	1	3
Herpes zoster	3	8		
Bacterial infection· non-specific	3	8	1	3
Nasopharyngitis	2	5		
Oral candidiasis	2	5		
Sepsis	2	5	2	5
Musculoskeletal, connective tissue and bone disorders				
Arthralgia	13	33	3	8
Myalgia	10	25	2	5
Bone pain	9	23	4	10
Back pain	7	18	1	3
Neck pain	5	13		
Pain in limb	5	13	2	5
Hematologic disorders				
Leukocytosis	20	50	1	3
Anemia	8	20	2	5
Thrombocytopenia	7	18	5	13
Febrile neutropenia	5	13	3	8
Neutropenia	4	10	4	10
Disseminated intravascular coagulation	3	8	3	8
Lymphadenopathy	3	8		
Vascular disorders				
Hypotension	10	25	2	5
Flushing	4	10		
Hypertension	4	10		
Pallor	4	10		
Psychiatric disorders				
Anxiety	12	30		
Depression	8	20		
Agitation	2	5		
Confusion	2	5		
Ocular disorders				
Eye irritation	4	10		
Blurred vision	4	10		
Dry eye	3	8		
Painful red eye	2	5		
Renal and urinary disorders				
Renal failure	3	8	1	3
Renal impairment	3	8		
Oliguria	2	5		
Incontinence	2	5		
Reproductive system disorders				
Vaginal hemorrhage	5	13		
Intermenstrual bleeding	3	8		
Ear disorders				
Earache	3	8		
Tinnitus	2	5		

[OVERDOSAGE]

If symptoms suggestive of serious Arsenic Trioxide toxicity (e.g., convulsions, muscle weakness and confusion) appear, Arsenic Trioxide should be immediately discontinued and chelation therapy should be considered. A conventional protocol for acute Arsenic Trioxide intoxication includes dimercaprol administered at a dose of 3mg/kg intramuscularly every 4 hours until immediate life-threatening toxicity has subsided. Thereafter, penicillamine at a dose of 250mg orally, up to a maximum frequency of four times per day (≤ 1 gm per day), may be given.

[PACKAGE]

Each 10 ml Asadin vial contains 10 mg of Arsenic Trioxide. 10ml per vial.
1 vial per box.

[STORAGE]

Do not use after expiration date printed on the label.
Please store below 30°C and protect from light.
Please keep out of reach of children.

[MANUFACTURER]

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[IMPORTER AND MARKETING AUTHORIZATION HOLDER IN MALAYSIA]

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[DATE OF REVISION]

28 May 2025